

The peripartum brain

Pregnancy turns the cerebral circulation pro-thrombotic



Clotting is dialled up to limit blood loss at delivery — the same shift clots the brain.

Highest risk: late 3rd trimester
→ first 6 weeks postpartum

Stroke in pregnancy

16–41 / 100,000 pregnancies
Hypercoagulability + venous stasis; peaks late & postpartum

Pre-eclampsia / eclampsia

Hypertension + end-organ injury
Endothelial injury → seizures, stroke, hemorrhage · PRES

RCVS

Thunderclap headache
Segmental 'string-of-beads' narrowing; resolves in weeks

Cerebral venous thrombosis

Sinuses clot in the peripartum window
Treat: anticoagulation (LMWH)

Hormones & lifelong risk

Estrogen tunes the vessels; pregnancy forecasts the future

Migraine with aura

Aura = independent stroke risk

Magnified by estrogen-containing
contraception

Absolute contraindication to CHC

Pregnancy = stress test

Gestational diabetes → ~50% in 5 yr

Gestational HTN → 2–4× later HTN

Pre-eclampsia → ≥ 2× stroke later

A decades-long window to act

Atrial fibrillation

Higher stroke risk than men

6.2% vs 4.2% per year (NVAF)

Female sex adds a risk point

Worse deficits & outcomes

Lupus & APS

78% of autoimmune disease

is in women; lupus 7× more

Antiphospholipid syndrome &

Libman–Sacks → stroke